GOALS OF CARE — HOSPITAL PASSPORT

CONFIDENTIAL

✔ FOR FULL CODE RESUSCITATION

John does NOT have a Non-For-Resuscitation (NFR) plan. He is for FULL resuscitation.

PARTICIPANT DETAILS

Full Name	John Eric Ross	**Preferred Name**	John
Date of Birth	30/12/1961 (Age 64)	**NDIS Number**	574 479 496
Address	59 Severn St, Epping VIC 3076	**Phone**	0488 700 146

KEY CONTACTS

NOK (Wife)	GP	Support Coordinator	Pharmacy
Beverly Ross 0416 104 301	Dr Shahzad Ahmed Plenty Valley Medical 03 9436 9677	Gulsun Ali Interconnection 0407 346 180	Terry White Pharmacy South Morang 03 9408 3222

MEDICAL OVERVIEW

Primary Dx	Severe Diabetic Peripheral Neuropathy — sensorimotor impairment, reduced limb function, chronic pain, weakness, fatigue
Secondary Dx	Type 2 Diabetes | End Stage Kidney Disease (haemodialysis) | Bowel & bladder incontinence | SPC in situ | Lower limb & abdominal wounds | Pressure injury risk
Mobility	Non-ambulant. Transfer requirements currently under review by allied health team. **Floor hoist: X3 staff. Ceiling hoist: X2 staff.**

REGULAR DIALYSIS SCHEDULE

Location	Days	Time	Transport
Northern Hospital (Epping Campus)	Every Tuesday, Thursday and Saturday	**2:30 PM**	Ambulance Victoria scheduled transport

DECISION-MAKING CAPACITY

John is **cognitively capable** of making his own decisions regarding care and treatment. No communication deficits. His wife, **Beverly Ross (NOK — 0416 104 301)**, should be contacted for significant medical decisions or hospital admissions.

INSULIN & MEDICATIONS

Insulin Administration	Before Breakfast: **16 units Toujeo + 5 units Novorapid** Before Lunch: **5 units Novorapid** Before Dinner: **5 units Novorapid**
BGL	Blood glucose checks **3x daily**. Cannot self-monitor (reduced finger sensation).
Medications	Full support required for medication admin — cannot manage packaging. Refer to medication chart.
BOC	John frequently declines to take his medications when presented by staff. He will request that medications be left on the table, stating he will take them later. John then often dozes off and forgets to take them. When staff follow up or remind him, he becomes agitated and verbally refuses, directing staff to leave him alone. This pattern occurs across all medication rounds, not only in the morning. On dialysis days, John tends to take his breakfast and morning medications late, which results in him skipping his lunch medications. Refer to Dignity of Risk — Medication Self-Administration Behaviour document (OLF-DOR-JR-001) for staff response guidelines.

PRE-TRANSFER CHECKLIST (Staff to complete before ambulance departure)

■ Breakfast	■ Morning Insulin	■ Morning Medications
■ Lunch	■ Lunch Insulin	■ Lunch Medications
Date: _____	**Time:** _____	**Staff:** _____

TRANSFER INFORMATION

Hoist Requirements	**Ceiling Hoist: X2 staff | Floor Hoist: X3 staff**
Reason	Haemodialysis 3x/week at Northern Hospital. Non-ambulant — requires ambulance with hoist/stretcher.
BP Monitoring	John has an unstable blood pressure condition. BP can rise or drop rapidly. **Blood pressure must be checked and monitored before any attempt is made to transfer him out of bed**, as sudden changes may place him at increased risk during transfers.
Transfer Review	John's transfer requirements are currently under review by his Occupational Therapist (OT) and Physiotherapist. The hoist and other transfer equipment are currently being trialled and assessed by his treating allied health team. **Support workers must not transfer John out of bed using the hoist or any other transfer equipment until written clearance and specific transfer instructions have been provided by his treating OT and Physiotherapist.**
Equipment	Hoist sling, Hospital Passport folder, medication chart.

Until clearance is received, staff must follow the current transfer plan and seek guidance from the Clinical Team or Management if there are any concerns. **The safety of John and staff must remain the highest priority at all times.**